



POLICY

Clinical Supervision

Scope (Staff):	All clinical staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

To promote high standards of clinical care and safe, consistent clinical practice across the Child and Adolescent Mental Health Service (CAMHS) as well as to provide support to CAMHS clinical staff in their profession within the clinical environment.

Risk

- CAMHS may contravene discipline specific supervision standards.
- Staff wellbeing and retention may be affected.
- Clinical skills and staff professional development needs will not be met.
- Poor client outcomes and experiences

Background

All CAMHS clinical staff require supervision relevant to their role and experience in line with this policy.

The purpose of clinical supervision is to;

- provide ongoing learning and clinical skill development;
- provide staff with a safe and supportive environment in order to cope with the demands of clinical work;
- promote reflective practice and maintenance of professional and ethical standards; and
- promote evidence-based practice.

Out of Scope

Clinical supervision is one component of professional development and support for CAMHS staff engaged in clinical work. For some disciplines it can also be a professional board requirement for maintaining registration. Other components not addressed in this policy include, but are not limited to:

- **Clinical Incident Debriefing:** A structured session for the group of people involved in a clinical incident with the aim of reducing impact and providing information on the availability of appropriate professional support. For more information see the [Responding to Potentially Traumatic Events](#) policy.
- **Cultural Consultation:** Encourages non-Aboriginal clinicians to maintain proficiency and reflect on their work practice and relationship with Aboriginal people, their carer's, families and communities. It is recommended particularly for non-Aboriginal clinicians who regularly work with Aboriginal people, their carer's, families and communities.
- **Cultural Supervision:** A group session facilitated by a senior Aboriginal clinician that provides Aboriginal Mental Health Workers an opportunity to discuss cultural and clinical concerns in a culturally safe space. For more information refer to the [Aboriginal Mental Health Service Guideline](#).
- **Education and Training:** The [CAMHS Learning and Development](#) page on HealthPoint provides staff with a central point in which to find out about training that is available including mandatory training.

- **Mentoring:** Creating relationships where productive conversations and learning can occur between two colleagues, while providing an opportunity to reflect on career challenges, discuss strategies and celebrate achievements. For more information visit the [CAHS Mentoring Program](#) page on HealthPoint.
- **Multidisciplinary Team Clinical Reviews:** A formal process to monitor clinical assessment and treatment progress and provide an opportunity to identify any barriers to discharge early in the episode of care and plan interventions to address these barriers. For more information see the [CAMHS Multidisciplinary Team Clinical Reviews Policy](#).
- **Operational Supervision:** Also referred to as line management. It is the role that the line manager has in operational decision-making and in overseeing the work of the staff member to ensure they are operating within their scope of practice.
- **Peer Support Worker Supervision:** Peer Support Workers are provided with supervision from a clinician with relevant clinical knowledge and skills who is different from their line manager. The Supervisor supports the Peer Support Worker in their role in accordance with the relevant Peer Worker Work Instruction available on the [CAMHS Work Instruction page](#).
- **Performance Review:** Supporting employee performance and development is key to continuous learning and is an ongoing process to ensure performance is monitored, achievements are recognised, and development activities are undertaken. The outcomes of clinical supervision can be integrated into the performance review process or more frequently if required. For more information visit the [Supporting Employee Performance and Development](#) page on HealthPoint.
- **Professional Supervision:** When a staff member working in a clinical environment seeks the advice of the Consultant Psychiatrist or another senior clinician who has overall clinical accountability.
- **Regulation and Credentialing:** The Australian Health Practitioner Regulation Agency (AHPRA) supports the 15 National Boards that are responsible for regulating the health professions. The primary role of the National Boards is to protect the public and set standards and policies that all registered health practitioners must meet. National Board requirements for supervision to maintain registration are not uniform across disciplines and staff must check their individual registration requirements. The AHPRA supervised practice framework applies to certain decisions made by National Boards or to meet National Board registration standards, eligibility or suitability requirements, or as required by National Law. Disciplines not regulated by AHPRA are also considered in this policy for the purposes of clinical supervision within CAMHS. For further information on these positions please refer to the Appendices within this document and the CAHS [Credentialing for Self-Regulating Allied Health and Health Science Professionals](#) Policy.

Definitions

Clinical Supervision: is the process of two or more clinicians (supervisor/s and supervisee/s) formally meeting to reflect and review clinical situations with the aim of supporting the CAMHS staff member in their clinical environment. It is conducted regularly with a specific set supervision time. The supervision session time should be protected and prioritised by both the supervisee/s and the supervisor/s. Supervision should be conducted in an appropriate environment that facilitates patient care/case discussion, reflective practice, and the setting and monitoring of learning goals and objectives.

Types of clinical supervision are;

Individual: is one-on-one clinical supervision and may be done by telephone or videoconference.

Group: is facilitated by an appointed supervisor. The principles of one-to-one structured supervision are applicable in the group supervision context, including the development of a supervision agreement and/or a record of attendance register. Planning must occur prior to establishing a supervision group. Consideration must be given to;

- the composition of the group;
- the number of supervisees in the group;
- the skills, experience and individual attributes of the supervisees;
- whether the group is the most appropriate/feasible form of supervision; and
- whether the group supervision meet the needs of clinicians requiring supervision.

Cross discipline: is individual or group supervision with more than one professional discipline involved.

Treatment-specific: is individual or group supervision required in order to develop specific competencies and/or when the clinical supervisor has expertise or specialisation in a specific area that is needed in the clinical setting.

Peer: is a self-directed meeting of two or more clinicians to supervise each other's work. It requires a strong motivation and commitment from participants and a clear purpose and structure. Participating clinical staff must have had similar training and experience. A clinical supervisor should not be in the same peer supervision group as a clinician they are currently supervising individually.

Supervisee: any CAMHS clinician receiving clinical supervision.

Supervisor: is a person with the relevant clinical experience, knowledge and skills appropriate for clinical supervision.

Supervision agreement: is a formal document which outlines the parameters of the relationship including; the supervisor and supervisee's details and responsibilities; an agreement for confidentiality; the frequency of clinical supervision; and a review date

of the supervision agreement. A supervision agreement is required for all individual sessions of clinical supervision and should be reviewed every 12 months. A new agreement is required when the supervision arrangement changes. See [Appendix 1: Clinical Supervision Agreement](#) and [Appendix 2: Record of Attendance Register](#).

Principles

- All CAMHS clinical staff must undertake at least one type of clinical supervision as defined in this policy.
- It is the responsibility of the line manager to ensure that suitable arrangements for clinical supervision are in place for all CAMHS clinicians.
- A [Clinical Supervision Agreement](#) and [Record of Attendance Register](#) is required for all types of clinical supervision.
- Supervision should be conducted in an appropriate and safe setting that enables the supervisee to reflect on, evaluate and analyse their work practice and experiences.
- When reflective practice occurs in clinical supervision, it can be in relation to reflecting on day to day clinical practice; triggered by a challenging clinical encounter; or in anticipation of having to manage a complex situation.
- Reflective practice is an effective process to develop self-awareness and facilitate changes in professional behaviour and should be built into clinical supervision to ensure clinical excellence.
- Safe clinical practice must always be promoted during clinical supervision session and any issues that have the potential to impact on clinical standards of care and / or workplace safety must be identified and the line manager informed.

Frequency

- It is recommended that CAMHS clinicians have a minimum of 1 hour of clinical supervision per month.
- The number of clinical supervision hours required per month may vary pro rata to the supervisee's hours of employment and/or length of experience.
- The frequency of clinical supervision is determined by the line manager in collaboration with the professional lead, clinical lead and/or senior clinician.
- Supervisees are encouraged to discuss any additional supervision requirements with their line manager who can then consult with the professional lead, clinical lead and/or senior clinician.

Modes

- Clinical supervision can occur in a variety of settings including face-to-face, telephone or virtually and suitable times and venues should be considered to minimise interruptions.

Setting Goals

- At the commencement of the clinical supervision arrangement the supervisor and supervisee should set short, medium and long-term goals and timeframes for these to be achieved. These should be set out in the [Clinical Supervision Agreement](#).
- The process of clinical supervision is based on individual learning goals, professional requirements relating to clinical practice and the organisation/service requirements of the supervisee.

Confidentiality

- Disclosure of identifying patient information during clinical supervision sessions should be avoided wherever possible.
- As part of the Clinical Supervision Agreement, clinical supervisors agree to keep the content of sessions confidential unless disclosure is authorised or required by law.

Managing Conflict

- Through the clinical supervision process, conflict may occur between the supervisor and supervisee, so it is recommended that at the commencement of a supervision agreement, both parties should agree to how conflict will be managed and/or resolved.
- If conflict cannot be resolved, it may be appropriate to invite a third party of mediate however the supervisor and supervisee should agree on the third party at the commencement of the supervision agreement.

Review

- The [Clinical Supervision Agreement](#) should be reviewed every 12 months to assess if there are any issues to do with arrangements, venue or mode and whether the agreement needs to be modified.
- Effective clinical supervision will ensure the supervisee is aware of the progress they have made against the goals set and the opportunities for further improvement.

External supervision (supervision NOT provided by WA Health staff)

- Wherever possible CAMHS clinicians should obtain clinical supervision from an appropriate CAMHS clinical supervisor or participate in peer supervision.
- Where this is not possible, and/or additional learning goals are required, external supervision may become necessary.
 - This includes where there are unique and specialised aspects of the service Model of Care which requires treatment-specific clinical supervision as defined in this policy. For example, family-based treatment (Eating Disorder Service) and Mentalization Based Therapy (Touchstone) and others.
- An application form must be completed. See [Appendix 3 - Application Form for External Clinical Supervision](#).
- The Head of Service and Line Manager, in consultation with the Professional Lead, Clinical Lead or Senior Clinician, can negotiate on the engagement of an external supervisor for the supervisee.
- Approval from the Director is required prior to the appointment of the external supervisor.
- Once the relevant Director has approved the request, the Line Manager must email the invoice to CAMHS Finance. Costs must be charged:
 - to the cost centre of the service in which the clinician is employed; and
 - in accord with the [CAHS Procurement of Goods, Services and Works policy](#).
- The Line Manager, Professional Lead, Clinical Lead or Senior Clinician must ensure that the external clinical supervisor has the appropriate credentials prior to their engagement. This should include evidence of:
 - relevant professional registration or equivalent;
 - recent practice relevant to the proposed supervision;
 - adequate professional indemnity insurance; and
 - names of appropriate referees.
- It is the responsibility of the Line Manager, Professional Lead, Clinical Lead or Senior Clinician to verify the accuracy of information and evidence of supervision competence that is recognised by CAMHS.
- The evidence obtained from an external clinical supervisor should be retained by the Line Manager, Professional Lead, Clinical Lead or Senior Clinician in accordance with CAHS record-keeping requirements.

- External clinical supervisors are to maintain communications with the Line Manager and Professional Lead, Clinical Lead or Senior Clinician as identified in the [Clinical Supervision Agreement](#) .
- By entering into and signing a [Clinical Supervision Agreement](#) , external clinical supervisors agree to keep the content of sessions confidential unless disclosure is authorised or required by law.

Roles and Responsibilities

Line Manager

- Liaise with the Professional Leads, Clinical Leads and/or Senior Clinicians as well as governing bodies, where applicable, regarding clinical supervision requirements.
- Ensure supervisees are accessing suitable clinical supervisors, in consultation with the Professional Lead, Clinical Lead and / or Senior Clinician as required.
- Ensure supervisees are aware of this policy and the expectations.
- Assist supervisees to manage their time effectively in conjunction with existing workloads in order to participate in clinical supervision.
- Ensure issues brought to their attention in association with clinical supervision processes such as clinical risk and/or staff wellbeing concerns are dealt with promptly and appropriately.
- Keep a record of supervision agreements pertaining to supervisees they are managerially responsible for unless records are being kept by the Professional Lead, Clinical Leader and/or Senior Clinician.

Professional Lead / Clinical Lead / Senior Clinician

- Select appropriate clinical supervisors for individual and group supervision in collaboration with supervisees and their line managers.
- Endorse appropriate peer supervision groups.
- Maintain a database of clinical supervisors.
- Ensure clinical supervisors are appropriately skilled in supervision.
- Liaise with the line manager and supervisee where concerns regarding supervision are raised.
- Escalate any clinical risks or staff wellbeing concerns to the line manager.
- Provide support in the engagement of an external clinical supervisor as described in the process above.
- Keep a record of supervision agreements pertaining to clinicians of the relevant discipline unless records are being kept by the line manager.

Supervisor

- Must have completed professional supervision training requirements as it relates to each discipline or possess the relevant clinical experience, knowledge and skills appropriate for clinical supervision.
- Ensure availability as agreed in the [Clinical Supervision Agreement](#) .
- Ensure appropriate record keeping and confidentiality in line with this policy.
- Ensure the supervisory relationship continues in line with the [Clinical Supervision Agreement](#) including following up missed sessions and making arrangements while on leave.
- Escalate any staff wellbeing concerns to the line manager.
- If applicable, maintain their own professional development through active participation in a registered professional development program to meet their professional obligations as a supervisor.
- Promote safe clinical practice and ensure issues that have the potential to impact on clinical standards of care and / or workplace safety are identified, the line manager is informed, and an action plan is developed.

Supervisee

- Ensure availability as agreed in the [Clinical Supervision Agreement](#)
- Ensure adequate preparation of issues and cases to discuss with the supervisor.
- Ensure appropriate record keeping and confidentiality in line with this policy.
- Commit to the process of clinical supervision in line with this policy.
- Ensure the supervisory relationship continues in line with the [Clinical Supervision Agreement](#) including following up missed sessions and making arrangements while on leave.

Documentation requirements

- A [Clinical Supervision Agreement](#) is signed by both parties and forms the basis of the supervisory relationship. A copy must be kept by the supervisee and the Line Manager or the Professional Lead, Clinical Lead or Senior Clinician.
- A record of the group supervision sessions should be kept by both clinical supervisor and supervisee via a [Record of Attendance Register](#) .
 - There is an option to record the sessions as a service event in PSOLIS with notes kept separately by the supervisor and supervisee.
- Where an individual discipline has specific requirements for the maintenance of records of supervision content, these are to be adhered to.

References and related external legislation, policies, and guidelines
<u>AASW Supervision Standards and Professional Supervisor Directory</u>
<u>Australian College of Mental Health Nurses: Position Statement – Clinical Supervision for Nurses and Midwives.</u>
<u>ESSA Continuing Professional Development Guidelines</u>
<u>Exercise and Sports Science Australia: Continuing Professional Development (CPD) Points Guidelines</u>
<u>National Competency Standards for Dietitians in Australia</u>
<u>Occupational Therapy Board of Australia's Guidelines on Continuing Professional Development</u>
<u>Physiotherapy Board of Australia's Guidelines on Continuing Professional Development</u>
<u>Professional Standards (speechpathologyaustralia.org.au)</u>
<u>Psychology Board AHPRA Registration Standards</u>
<u>Psychology Board of Australia Guidelines for Supervisors</u>
<u>The Royal Australian & New Zealand College of Psychiatrists website</u>
<u>WACHS Allied Health Assistants Policy</u>
WA Health ANF industrial agreement (2024)
Related CAHS internal policies, procedures and guidelines <i>(if required)</i>
<u>CAHS Credentialing for Self-Regulating Allied Health and Health Science Professionals Policy</u>
<u>CAHS Responding to Potentially Traumatic Events</u>
<u>CAMHS Multidisciplinary Team Clinical Reviews Policy</u>
Useful resources (including related forms) <i>(if required)</i>
<u>Application Form for External Clinical Supervision</u>
<u>CAHS Mentoring Program</u>
<u>CAMHS Clinical Supervision Templates</u>

[CAMHS Learning and Development HealthPoint page](#)

[Supporting Employee Performance and Development](#)

This document can be made available in alternative formats on request.

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Healthy kids, healthy communities Compassion Excellence Collaboration Accountability Equity Respect			

Appendix 1: Clinical Supervision Agreement template

Suggested for individual, peer and one-on-one sessions

[*Click here to access the editable Word and writable PDF versions of this template](#)

Supervisor Name		Supervisor Position	
Supervisee Name		Supervisee Position	
Agreement Commencement Date:		Review Date:	

Type of Clinical Supervision

☐ Individual ☐ Group ☐ Peer ☐ Other

Goals of Clinical Supervision

Short term goals	
Medium term goals	
Long term goals	

Agreed approach for managing conflict	
Nominated 3rd party	

Record of sessions

Planned Date	Key area discussed	Mode of Contact	Completed

- ☐ We have read and understood the CAMHS Clinical Supervision Policy.
- ☐ We agree to actively participate in clinical supervision as outlined in this Agreement.
- ☐ We agree to keep the content of the clinical supervision sessions confidential (including maintaining client confidentiality), unless disclosure is authorised or required by law.
- ☐ We agree to conduct all clinical supervision sessions in accordance with CAHS Values.

Supervisee Name:

Supervisor Name:

Appendix 2: Template – Record of Attendance Register (suggested for any group sessions)

[*Click here to access the editable Word and writable PDF versions of this template](#)

Supervisor Name:	
Supervisor Position:	
Date:	

Supervisee name:	Supervisee position:

Key area/s discussed	Agreed actions	Timeframes

Agenda items for next session:

Appendix 3: Application form – External Clinical Supervision

[*Click here to access the writable PDF version of this template](#)

Wherever possible, CAMHS staff should obtain clinical supervision from an appropriate CAMHS clinical supervisor or participate in peer supervision. Where this is not possible and/or additional learning goals have been identified by the Head of Service, eligible staff may apply for external supervision.

Please ensure that you have discussed your clinical supervision needs with your Head of Service and/or Line Manager prior to completing this form.

CAMHS Employee Details

Full name:

HE number:

Position Title:

Service Name:

External Supervision Details

Reason for applying for external supervision:

Three key goals of external supervision:

1.

2.

3.

Name of external supervisor:

Position Title:

AHPRA Registration (if applicable):

Organisation (if applicable):

Number of sessions:

To be done during working hours: Yes / No

CAMHS Employee to email form to Service Manager and Head of Service

Approvals

Service / Line Manager Name:

HE number:

Position title:

External clinical supervision application approved: Yes / No

Date:

Head of Service Name:

HE number:

Position title:

External clinical supervision application approved: Yes / No

Date:

Service Manager to email form to Director ICAMHS East / South / North / Central / Inpatient Unit

Director:

HE number:

Position title:

External clinical supervision application approved: Yes / No

Date:

Completed form to be emailed to CAHSCAMHSProfessionalLeads@health.wa.gov.au

CAMHS Professional Lead Process

- ☐ Review external supervisor credentials
- ☐ Determine any Conflict of Interest or secondary employment requirements
- ☐ Forward Clinical Supervisor Agreement template & link to policy to employee
- ☐ Note 12-month review date

Appendix 4: Allied Health

Allied Health Assistants

In the context of the role of an Allied Health Assistant (AHA), supervision is used as a term to describe the monitoring process whereby a designated Allied Health Professional (AHP) directly observes the AHA conducting a clinical activity. Supervision is required in the following instances:

- when monitoring the performance of the AHA for safety and quality purposes;
- when determining the competence of an AHA to perform an activity;
- when providing immediate feedback and demonstration of aspects of an activity to improve performance;
- where professional regulatory bodies stipulate the level of supervision required for a clinical activity.

This is outside the scope of this policy however, CAMHS have received permission for staff to refer to the [WACHS Allied Health Assistants Policy](#) for more information.

Clinical Psychology

The continuing professional development (CPD) requirements for psychologists are set out in the [Continuing professional development registration standard](#). These include:

1. Developing a learning plan based on objective self-assessment
2. Completing 10 hours of peer consultation activities annually
3. Completing 20 hours of other CPD activities annually
4. Maintaining a CPD portfolio that includes the learning plan, activity log and reflection and submitting the portfolio to AHPRA within 28 days if selected for audit.

Board-approved supervisors are experienced psychologists who have undertaken Board-approved training in competency-based supervision. For more information see the [Psychology Board of Australia Guidelines for Supervisors](#).

Dietetics

The [National Competency Standards for Dietitians in Australia](#) are used to facilitate a shared understanding of competence.

As part of the performance criteria for professional practice, Dietitians must:

- consistently demonstrate reflective practice in collaboration with supervisors, peers and mentors (1.1.3).
- participate in supervision, teaching and mentoring processes with peers, students and colleagues (1.3.4).

For more information visit [Dietitians Australia](#).

Exercise Physiology

Under the Exercise and Sports Science Australia (ESSA) Continuing Professional Development (CPD) Program, accredited professionals must maintain and grow their competence – that is, their capacity to practice safely, effectively and legally within their evolving scope of practice.

ESSA accredited professionals are encouraged to undertake a broad range of CPD activities to meet the annual CPD points requirements for annual accreditation. Activities include mentorship and supervision.

For more information see the [ESSA Continuing Professional Development Guidelines](#) and the [ESSA website](#).

Occupational Therapy

Clinical supervision may be used for Continuing Professional Development (CPD) requirements. The [Occupational Therapy Board of Australia's Guidelines on Continuing Professional Development](#) suggest written reflections on day-to-day clinical practice and professional or interprofessional interactions as examples of CPD activities.

Physiotherapy

Clinical supervision may be used for Continuing Professional Development (CPD) requirements. The [Physiotherapy Board of Australia's Guidelines on Continuing Professional Development](#) suggests a variety of non-formal CPD learning activities including goal setting and reflection on possible changes to practice as a result of learning.

Social Work

The Australian Association of Social Workers (AASW) *Supervision Standards 2014* articulate expectations for the professional supervision of social workers in the current and varied contexts which social work is practiced in Australian.

For more information refer to; [AASW Supervision Standards and Professional Supervisor Directory](#).

Speech Pathology

The governing body for Speech Pathologists in Australia is Speech Pathology Australia. To be employed at CAHS CAMHS, you need to be able to demonstrate eligibility for certified practising membership of this governing body at the commencement of employment and throughout employment.

Speech Pathology Australia provides Professional Standards for Speech Pathologists in Australia (August 2020) which details the skills, knowledge and practises that Speech Pathologists need to maintain to practise as a Speech Pathologist. A critical domain in these standards is Domain 2 - Reflective Practice and Lifelong Learning which refers to engagement in reflective clinical supervision, mentoring and professional development that enhances ongoing standards of practise.

For more information refer to; [Professional Standards \(speechpathologyaustralia.org.au\)](https://speechpathologyaustralia.org.au)

In addition to this Speech Pathology Australia provides guidelines in the *Supervision Standards* (November 2014) and *Supervision FAQs* (October 2018) including recommended skills, experience, role and responsibilities of the supervisee and clinical supervisor. For more information refer to; [Supervision \(speechpathologyaustralia.org.au\)](https://speechpathologyaustralia.org.au)

Appendix 5: Medical and Nursing

Psychiatry

For more information regarding Continuing Professional Development (CPD) requirements refer to [The Royal Australian & New Zealand College of Psychiatrists website](#).

Professional supervision provided by, for example the Head of Department and/or Consultant Psychiatrist, is when an employee working in a clinical environment seeks the clinical advice from the person that has overall clinical accountability. This is different to clinical supervision as defined in this policy and is out of scope.

Mental Health Nurses

It is the position of the Australian College of Nursing, the Australian College of Mental Health Nurses and the Australian College of Midwives that Clinical Supervision is recommended for all nurses and midwives irrespective of their specific role, area of practice and years of experience.

For more information refer to the [Australian College of Mental Health Nurses: Position Statement – Clinical Supervision for Nurses and Midwives](#).

In accordance with the [WA Health ANF industrial agreement \(2024\)](#), Registered Nurses Level 1 are responsible, where applicable, for the clinical supervision of Enrolled Nurses and Assistants in Nursing. Registered Nurses Level 2 are responsible for the clinical supervision of Registered Nurses Level 1 and Enrolled Nurses. Clinical supervision is not provided by a professional who has organisational responsibility to direct, coordinate or evaluate the performance of the supervisee (i.e. the line manager as described as operational supervision).